



CLINICAL CASE STUDY FORMAT INTERNSHIP CLINICAL MENTAL HEALTH COUNSELING UNIVERSITI PENDIDIKAN SULTAN IDRIS

Client Profile

Client ID :	Date of Referral :
Date of Birth :	Referred By :
Age :	History of Diagnosis :
Gender :	Setting :
Race :	
Marital Status :	
Occupation :	

Reason for Referral

State the specific reason for the client's referral, including background details and the recommendations of the referring individual or organization.

Example: *"The client was referred by [Referrer Name/Organization] for [specific concern, e.g., treatment of anxiety and depression] after reporting persistent symptoms of low mood, intrusive thoughts, and lack of motivation."*

Introductory Statement

A brief sentence orienting the focus of the case and illustrating its significance or complexity.
Example: *"This case illustrates the importance of managing suicide risk in a middle-aged man with chronic depression."*

Presenting Complaint

Detail the client's main complaints in their own words, supplemented by functional inquiry.

- **Onset, duration, and progression of symptoms:**
- **Precipitating or mitigating factors:**
- **Severity of symptoms:**
- **Impact on daily functioning:**
- **Client's own understanding or attribution of symptoms:**

Example: *"The client reports persistent feelings of sadness and guilt for over six months, stating, 'It feels like I can't escape this dark cloud.' Symptoms have intensified since losing his job and are affecting his sleep, appetite, and social relationships."*

History of Presenting Complaint

Expand on the presenting complaint with a detailed history. Include:

- Symptom history (onset, triggers, timeline).
- Functional inquiry (e.g., sleep, energy, concentration, appetite, mood).
- Cultural context of symptoms and illness.
- Impact on work, relationships, and personal life.

Past Psychiatric History

Summarize the client's previous mental health issues, treatments, and outcomes.

- Previous diagnoses or symptoms.
- Psychiatric treatments (medications, therapy, hospitalizations).
- Self-harm or suicidal ideation history.
- Response to treatments.

Medical History

Include relevant physical health conditions that may interact with mental health symptoms.

- Chronic illnesses.

- Recent surgeries or hospitalizations.
- Current medications (non-psychiatric).

Family History

Focus on mental health history, relationship dynamics, and familial influences.

- Mental health issues in the family.
- Family dynamics and relationships.
- Cultural or religious influences.

Social History

Describe the client's social circumstances and relationships.

- Living arrangements.
- Employment/financial status.
- Support network.
- Social and recreational activities.

Mental Status Examination (MSE)

Provide observations of the client's behavior and psychological functioning.

- **Appearance and behavior:**
- **Speech:**
- **Mood and affect:**
- **Thought process and content:**
- **Perception:**
- **Cognition:**
- **Insight and judgment:**

Risk Assessment and Psychological Assessment Results

Evaluate any potential risks to the client or others.

- **Suicide risk:** Thoughts, intent, plan, access to means.
- **Self-harm risk:** History or current behavior.
- **Harm to others:** Aggression, violent tendencies.
- **Other risks:** Neglect, substance misuse.

Report any psychological assessment results

Case Formulation

Summarize the case using the **4 P's framework**:

- **Predisposing factors:** Background factors contributing to vulnerability.
- **Precipitating factors:** Events triggering the current episode.
- **Perpetuating factors:** Factors maintaining symptoms.
- **Protective factors:** Strengths and supports mitigating risk.

Provisional Diagnosis

Provide a clinical diagnosis using the **DSM-5** or **ICD-11** criteria. If uncertain, list provisional diagnoses.

Differential Diagnosis

Is it clear enough the diagnosis? Management addressing differential diagnosis includes reviewing old notes both treatment history and medical, obtaining a thorough longitudinal history from the individual and meeting with family members for clarification

Treatment Plan

Outline interventions and recommendations.

- **Short-term goals:** Immediate priorities (e.g., safety, symptom stabilization).
- **Long-term goals:** Broader recovery objectives.
- **Interventions:** Therapy type, medications, psychoeducation, lifestyle changes.

Progress and Outcomes

For ongoing cases, detail the client's response to treatment and any changes in symptoms or functioning.

Ethical Considerations

Highlight any ethical issues or challenges encountered in the case.

Conclusion and Reflection

Summarize key learnings from the case and its broader implications for mental health practice.